Case Presentation: Paediatrics

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8th sem

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Patient Particulars

- Name- Mr. TS
- Age- 7yrs
- Sex- Male
- Address- Jagiroad, Assam
- Religion- Hindu
- Source of Information- Child and the mother (Reliable)
- Age- 32yrs
- Educational status of mother- 10th passed
- Occupation- Homemaker
- Occupation of the Father- Police
- DOA- 22/10/19
- DOE- 22/10/19

Chief Complaints

- Fever for last 7 days

History of Present Illness

- The child is having fever for the last 7 days. It is sudden in onset, Persists throughout the day, high grade of 103.6 deg F, associated with chills and rigor, relieved on the use of occasional over the counter drugs, however after a relief of few hours the fever spikes up back again. Other than that there are no relieving factors.
- There is associated malaise and decreased appetite with easy fatigability.
- There is no diurnal variation of fever

- No complaints of altered sensorium, abnormal body movement, headache, neck rigidity, vomiting,
- No rashes, cough, difficulty in breathing, sore throat, ear ache or hearing difficulty
- No complaints of yellowish discoloration of eyes, skin or urine, abdominal pain, loose stools, burning micturition or other urinary symptoms.
- No history of injuries, bone pain or joint pain.
- Appetite and thirst is decreased, but no significant history of weight loss. Frequency of bowel movement has decreased however there is no change in consistency or bloody nature. Bladder habits are normal

Past History

- No history of such long duration fever in the past.
- No previous significant medical or surgical history. (Epilepsy, hepatitis, Asthma, Tuberculosis, Malaria, Kala azar, Typhoid fever)

Family History

- Non consanguineous Marriage
- No history of similar complaints in the family.
- No history of recent infections in the family.
- No history of tuberculosis contact.
- No history of infectious disease outbreak in the community (Malaria, JE, Dengue, Kala azar etc)

Perinatal History

- Antenatal:

Uneventful

Mother is negative for HIV, HepB and syphillis on regular testing

No maternal history of fever, rash, blurring of vision, headache, bleeding PV, Discharge, pedal edema.

USG normal scan

No consumption of drugs other than those prescribed by physician

No exposure to radiation

Regular supplementation of folic acid later added iron and calcium

- Natal

Normal Vaginal delivery at term with no complication.

Baby cried immediately after birth

No jaundice

Urine and meconium passed less than 24 hrs after birth

No NICU admission history

Breast feeding started within 1 hr after birth

Birth doses of Vaccines were given after birth (BCG,HepB, OPV-0)

- Neonatal

Exclusive breast feeding till 6m and later continued for 1 yr along with supplementary food.

No history of any significant medical disorder during the infancy.

Developmental History

- The Mother couldn’t specify much regarding developmental history, however she mentions the parameters were reached along with other babies of similar ages in the community.
- Patient has achieved developmental milestones according to age.
- Normal scholastic performance of the child

Immunization History

- Taken according to the schedule (NIS)
- As mentioned, birth doses were taken
- Last vaccine taken was DTP at last yr

Nutritional History

- Typical dietary pattern:

Breakfast: Milk 1 glass (166kcal;8g)
Biscuits 3-4 nos. (40kcal;4g)
Lunch: Rice 2 katori (232kcal; 6g)
Veg. 1 katori (56kcal; 2g)
Dal 2 katori (172 kcal; 6g)
DinnerRice 2 katori (232kcal; 6g)
Veg. 1 katori (56kcal; 2g)
Dal 2 katori (172 kcal; 6g)
Nonveg: Chicken/Fish/ egg 2 serving every alternate days
Snacks: bread Or Fruits or Processed snacks two times a day
Total Calorie and Protein intake of regular substances : (1626kcal; 56g)
Age specific requirement: (1690; 29.5)
No protein deficit; mild calorie deficit of approx 4 %

Drug and allergy history

- Other than OTC antipyretics no other drugs was taken by the child
- Child is not allergic to any known Ingestant Inhalant or Contactant

SocioEconomic History

- Nuclear family of 3 members: Father sole earning member.
- Father works in Police. Overall lower middle class family (Kuppuswamy scale)
- Pucca house, Pucca and Sanitary latrine.
- Source of Drinking water: Tubewell. Mostly filtered but not always.
- Source of Cooking fuel: LPG cylinders
- History not suggestive of overcrowding, poor lighting or indoor air pollution.

Summary

- A 7yr old boy, born to a non consanguinous married couple, belonging to Lower middle class background, with normal nutritional immunization and developmental history, presents with an acute onset fever of 7 days with chills, rigor, and decreased bowel movements persisting throughout the day, but with no rashes or signs of localization.

EXAMINATION

- Child is alert conscious oriented to time place and person, but a bit irritated , tired and uncooperative due to his ailment

Vitals:

Body temp: 101.4 deg F (Oral)

RR: 18 breaths per minute. Regular. Normal in depth. Abdominothoracic in type

Pulse: (Right Radial Pulse) 86 bpm, regular in rhythm, normal character, raised volume, no radioradial or radiofemoral delay. Other peripheral pulses are bilaterally and equally palpable.

BP: 120/80 mmHg (Right arm supine position)

Contd

- Head to toe:

Hair- normal in colour, no easy pluckability

Face appears normal

Eyes- No congestion

Pallor- Absent

Icterus- Absent

Ears- Normally placed, no pain on palpation near the ears

Nose and PNS- No discharge or signs of inflammation

Oral Cavity- No sign of local inflammation or infection.

Cyanosis- Absent

Tongue is coated.

Neck nodes- Non palpable

Thyroid- Not palpable

Neck veins- Not rasied

Neck movements – Normal

Skin- No rashes, local inflammatory signs.

Hand and nails- Normal

Clubbing- Absent

Oedema- Absent

Chest and Spine- Normal

Skin Pinch test- Normal

Lower limbs- Normal

Anthropometry

Parameter	Value	Interpretation	Comment
Height for age	122cm	Med- +1s.d	Normal
Weight for age	21.4kg	-1S.d - Med	Normal
BMI	14.37kg/m^2	-1S.d - Med	Normal

Systemic Examination

- Findings:
- Abdomen:

Inspection: Normal shape, Umbilicus centrally placed and inverted, movement with respiration symmetrical, no local signs of inflammation, lump, pulsation, discoloration, scar, distended veins.

Palpation: Superficial: Inspectory findings confirmed. no local rise of temperature (more than other areas) or tenderness.

Deep: Liver is palpable(around 2cm below R subcostal margin along MCL) . Non tender, firm, smooth surface, round border, non pulsatile, moves on respiration. Upper border felt at 5th Icspace.

Spleen and kidneys were not palpable

Percussion: Tympanic

Auscultation: Bowel sounds were heard.

Groin and hernial sites were normal

PR was not done

- CNS: WNL (No disturbance or neurological deficit was found)
- CVS: WNL (heart sounds 1st and 2nd heard. No s3 or s4, murmur.)
- Resp: WNL (Bilateral and equal entry on both lungs. Vesicular breath sounds. No added sounds.)

Provisional Dx

- Probable case of enteric fever causing a fever of short duration with no complications
- Diff dx: Acute viral hepatitis, UTI, Malaria Dengue, Kala azar, JE, JIA, Infectious mononucleosis, Kawasaki disease